

إسم المريض ربيعاً : سهير محمد حسين احمد
المحافظة : القاهرة الحي / المدينة : الازهر
رقم البطاقة : 27312222101623 رقم التليفون : 01142132781
العمر(تاريخ الميلاد) : 22-12-1973 المهنة : بدون عمل
العنوان : ~ تاريخ تسجيل الإستمارة : 19-09-2026

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التعليق على التشخيص : -----

الاسم: د/محمد كيلاني
 الاسم: د/ندى حسن
 الاسم: د/نرمين رمضان
 الوظيفة: _____
 الوظيفة: _____
 الوظيفة: _____
 التوقيع: _____
 التوقيع: _____
 التوقيع: _____
 تعتمد صحة البيانات الخاصة بالمجالس
 وتلتزم جهة العلاج بلائحة المجالس الطبية.
 كما تقر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
 رئيس اللجنة ومدير
 المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

----- / د ----- / د ----- / د. | ----- / د. |

التشخيص النهائي :
 خطة العلاج :
 المستشفى :
 قسم :
 جهة العلاج :
 ملاحظات :
 نفقات :

رئيس
المجلس الطبي المتخصص

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Name: سهير محمد حسين احمد National ID: 27312222101623	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Bone scintigraphy
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



جمهورية مصر العربية
بطاقة تحقيق الشخصية

سهير
محمد حسين احمد
قرية البراجيل
مركز أوسيم - الجيزة

١٩٧٣/١٢

٢٧٣ ١٢ ٢٢ ٢١ ٠ ١٦ ٢٣

LT2414991

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١١٥٤٦٧٣٠١٢



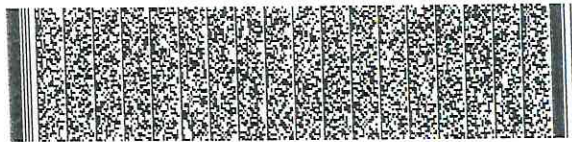
٢٠٢٦/٠٥ ٢٧٣١٢٢٢٢١٠١٦٢٣

بدون عمل



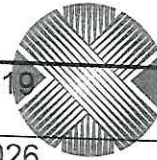
أنثى
مسلمة
متزوجة
صابر اسماعيل السيد على

البطاقة سارية حتى ٢٠٣٣/٠٥/٠٥



Imaging report

Pat. Name: Sohair Mohamed	Pat. ID: 3618519
Procedure Name: MRI Breasts	Date: 06/08/2026



كايرو سكان
للأشعة والتصوير
CAIRO SCAN
Radiology & LABS

MR MAMMOGRAPHY OF BOTH BREASTS:

CLINICAL SUMMARY:

Age: 50yrs

Right breast pain and swelling

IMPRESSION:

LEFT BREAST: (BIRADS 1)

Normal study

RIGHT BREAST: (BIRADS 5)

Multi focal malignant mass with skin thickening and level II pathologically enlarged axillary LN.

RECOMENDATION:

For histopathological evaluation

TECHNIQUE:

Multiplanar imaging was performed on a 1.5T magnet with a dedicated breast coil, using the following frequencies:

- Axial T1, T2 IR
- Sagittal T2
- Subtraction of pre & post contrast 3D dynamic axial fat suppressed FLASH sequence 1.5 mm slice thickness after IV injection of Gd-DTPA (0.1 mmol/kg) with MIP image reformatting

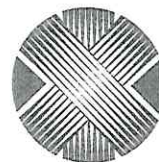
FINDINGS:

The breasts are composed of heterogenous fibroglandular tissue and fat. There is mild background parenchymal enhancement

LEFT BREAST:

- T2WIs show no areas of abnormal signal intensity.
- After injection of contrast, no rapidly enhancing masses or other suspicious enhancement patterns were observed to suggest invasive malignancy.
- Hence, no Time -intensity enhancement curves were done.
- Normal skin / nipple / areolar complex.
- No pathological axillary lymph nodes.

Imaging report



كايرو سكان
للشعة والتصوير
CAIRO SCAN
Radiology & LABS

Pat. Name: Sohair Mohamed Hussein	Pat. ID: 3618519
Procedure Name: MRI Breasts	Date: 06/08/2026

RIGHT BREAST:

- The middle aspect of the breast is the seat of two large speculated outline masses , 2 cm apart . The largest measures 5x4.6 cm reaching out to the nipple causing its retraction. The smaller measures 2.5x2.5 cm. They both elicit low T1 & T2 SI and high STIR SI
- After injection of contrast, they both showed dense contrast uptake.
- Diffuse skin thickening is noted.
- Level II pathological enlarged axillary lymph nodes are noted.

Best Regards
DR. DORRIA SALEM
Prof. of Radiology, Cairo University

For any inquiries regarding the findings in the report please contact us on
radreport@cairoscan.com.eg

Patient Name	سهر محمد حسين	Case No.	361786
Sex/Age	Female 52 Y	Patient No.	26467
Registered Date	31/8/2026 06:30:17 PM	Referred By Dr.	Interventional Radiology
Validation Date	17/9/2026 11:01:21 AM	Branch	Gustave Roussy
Reporting Date	17/9/2026 01:07:15 PM	Pathology No.	2769/26

Pathology Report

Clinical Data: Right breast mass (BIRADS 5).

Nature of Specimen: Tru-cut biopsy, collected on 31/08/2026, Cold ischemia time is 20 seconds.

Gross Examination:

-Four greyish, white tissue cores, ranging in length from 0.4 cm & 1.5 cm, totally submitted.

Technique:

- Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination:

- Histologic type: Invasive duct carcinoma.
- Histologic grade: 3 (differentiation 3, pleomorphism 3, mitosis 2).
- DCIS: Absent.
- Microcalcification: Absent.
- Angioinvasion: Absent.
- Perineural invasion: Absent.
- Tumor infiltrating lymphocytes (TILs): Moderate.

Results:

* IPT (All controls showed appropriate reactivity):

- ER: Negative nuclear reaction in tumor cells (score 0/8).
- PR: Negative nuclear reaction in tumor cells (score 0/8).
- Her2: Strong complete membrane reaction in more than 10% of tumor cells (score 3+).
- Ki67%: 25%.

Conclusion:

Right breast mass, Tru-cut biopsy:

-Invasive duct carcinoma, Grade 3.

IPT:

- ER: Negative (score 0/8).
- PR: Negative (score 0/8).



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر الجديدة - 11535 - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Nisar El Qadema - Cairo, Egypt

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- Her2: Positive (score 3+).
 - Ki67%: 25%.

****Scoring system: "ER, PR"**
 ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

**** Scoring system: "Her2"**
 Reference: ASCO-CAP Her2 Test Guideline Recommendation. IHC 0 (Null): as defined by no staining observed. IHC 0+ (Ultra-low) as defined by membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
 IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ibrahim Ghoneim, MD.



عرة رقم (4) - النور (الزل) - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
 Room No. (4) - First Floor-Building No. 987- Nile Corniche - Mizr El Qadima - Cairo, Egypt

naging report



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Imaging report

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Procedure Name: MRI Breasts	Date: 06/08/2026



CAIRO SCAN
Radiology & MS

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